EXECUTIVE BRIEF | CANDIDATE WORK SAMPLE

Should Intermountain back healthcare workflow AI?

ReferralIQ | Public-information review and illustrative underwriting | 10 October 2026

RECOMMENDATION: Advance Silna to diligence, validate current Notable overlap, benchmark Tennr and Rhyme, and defer both production deployment and any equity commitment.

The investment logic

Prior authorization digitization has a regulatory catalyst, but the investable question is whether a differentiated vendor can lower total cost and improve patient access better than the incumbent stack. Intermountain previously announced a Notable partnership [S31], raising the bar for a new platform.

Financial findings: hypothetical scenarios, not company forecasts

Modeled output	Downside	Base	Upside
90-day pilot net cash	($66,636)	($39,834)	($31,610)
Scaled annual run-rate net cash	($150,384)	$25,776	$347,400
First scaled year net cash	($196,950)	($31,268)	$249,500
Six-year gross venture MOIC	0.32x	2.97x	13.50x

Base economics: fully adopted recurring benefits cover ongoing costs by 1.21x. Annual cash conversion must reach 41.3% simply to break even. The first scaled year remains negative because of implementation and adoption ramp. A 90-day pilot should validate quality and economics, not promise immediate payback.

Investment committee gates

- Confirm incumbent technology, integration costs, and actual workflow eligibility before piloting.
- Measure verified minutes, errors, appeals and real cash reductions, not just staff capacity.
- Obtain startup ARR, margin, retention, customer references, cap table and actual deal terms before valuing equity.
- Require clinical, security and procurement independence before any vendor recommendation.

Included deliverables: 14-sheet Excel model with editable scenarios and formulas; detailed investment memo; machine-readable model contract and validation tests; Phase 3 developer handoff.